



Armadale Kalamunda Group

Dietetic Inpatient Prioritisation and Assessment Guideline

1. Purpose

The Dietetic inpatient prioritisation and assessment guideline has been developed by the Dietetics & Nutrition Department.

This guideline is designed to ensure patients are seen within a timely manner based on their level of acuity and referral reason. This allows better organisation and adequate patient care across the department.

2. Applicability

This guideline applies to dietitians at Armadale Health Service (AHS) providing care for patients with nutritional needs.

3. Guidelines

The Nutrition and Dietetics Department require guidelines to assist with the management of inpatients including prioritisation of both new and current patients. Dietitians use this guideline to prioritise workload based on the patient's presenting complaint and medical condition.

These guidelines are to be used to advise the dietitian of the procedure when seeing an inpatient. This includes assigning patients to an appropriate category based on the acuteness of the case. The assigned category will determine in what period of time the patient should be seen by the dietitian. The Dietetic Inpatient Prioritisation & Assessment guideline does not replace clinical judgement or existing clinical pathways and are to be used in times of staff shortages and high activity periods.

3.1 Dietary assessment and intervention process

3.1.1 Acceptance of referrals

- Referrals are accepted for any medical condition which requires immediate medical nutritional therapy. Referrals are received from medical, nursing and allied health staff verbally, via the electronic Patient Journey Board (PJB), via phone/pager, and during team meetings. Referrals should be documented in the medical notes. Patients will be seen according to the inpatient prioritisation guidelines refer to [Appendix 1](#) for adults and [Appendix 3](#) for paediatrics.
- Screening for patients requiring dietetic input can be conducted via iSOFT however should be limited to:
 - Known to AHS outpatient clinics and on home enteral nutrition (HEN).
 - Seen at AKG within last 12 months with malnutrition or at risk of malnutrition within the past 6 months and/or on HEN.

- Patients NBM/fluid diet for more than 48 hours (e.g. bowel obstruction, reduced GCS, dysphagia).
- Diverticulitis as admission reason, not previously educated.

3.1.2 Dietary intervention

On receiving a referral, the following procedures for dietary intervention are to be followed:

- Investigate the presenting case including diagnosis, history of presentation, past medical history, social history, medications, presenting dietary issues, biochemistry, and anthropometric data.
- Consult with appropriate staff to gain more information about the referral and/or determine the level of dietary intervention required or appropriate at the present time.
- Assess the presenting dietary issue(s), consulting the patient, and/or carer as appropriate, and develop a management plan.
- Discuss the dietary plan with appropriate nursing and/or medical staff and inform them of any specific dietary requirements.
- In some cases it may be necessary to contact the patient's Medical Officer to ensure that implementation of the dietary management plan is in keeping with the patient's care plan. This contact can be made via the team meeting, a written entry in the main body of the case notes, or by paging the Medical Officer directly.
- Document the nutritional assessment, diagnosis, intervention and monitoring plan in the medical notes using the Assessment Diagnosis Intervention Monitoring and Evaluation (ADIME) format. For further information on ADIME documentation refer to the International Dietetics & Nutrition Terminology (IDNT) folder on the w-drive. ADIME templates to assist with Digital Medical Record entries are available on w-drive, W:\Dietetics & Nutrition\AKH\Clinical Resources\DMR.
- Order any special dietary products if required.
- Advise Catering via iSoft of any therapeutic dietary requirements, refer to [AKG Therapeutic Diet and/or Fluids Procedure](#) and [Dietetics Prescribing High Energy High Protein Diets Guideline](#) details on ordering therapeutic diets.
- Update progress on the electronic PJB as appropriate. Green – ready for discharge, Yellow – requires review prior to discharge, Red – not safe for discharge.
- During admission, monitor the patient's progress and alter the dietary management as required.
- Refer to [Appendix 2](#) for prioritisation guidelines for review adult inpatients and [Appendix 4](#) for paediatrics.
- Complete the dietetic clinical handover for transfer between wards in iSOFT HI Nutr&DietDept, refer to [AKG Clinical Handover Procedure](#) and Dietetic Clinical Handover Guideline for details on completing handover.
- If required, initiate referral/s to other health professionals or community agencies, in consultation with the patient's medical officer, patient/family/carers.
- Ensure adequate discharge planning where appropriate i.e., organise nutritional product discharge supply script, transfer summary if going into residential care, referral to an appropriate dietetic service for continued care and support, relevant details entered into the patient discharge summary.

- Ensure that consent is obtained from patient and/or next of kin prior to sharing of information to external agencies e.g. care providers, private dietitian's, general practitioners, as per the [Information Access, Use and Disclosure Policy](#).

4. Supporting information

The following documents support the implementation of this policy:

- [Therapeutic Diet and Fluids Procedure AKG-PRO-0436-18](#)
- [Dietetics Prescribing High Energy High Protein Diets Guideline ALL-GUI-0542-20](#)
- [Red Mat System Guideline AKG-GUI-0501-19](#)
- [Dietetics Adult Malnutrition Diagnosis and Documentation Procedure AKG-PRO-0126-18](#)
- [Clinical Handover Procedure AKG-PRO-0521-19](#)
- [DOH Information Access, Use and Disclosure Policy MP 0015/16](#)

5. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 Comprehensive Care

6. Glossary

The following definitions are relevant to this policy.

Term	Definition
Dietary intervention	Used to describe any of the following procedures: nutritional assessment, nutritional monitoring, design and/or implementation of a therapeutic diet, enteral/parenteral feeding or any other direct time spent on the nutritional care of a patient.
New referrals prioritisation	Applies to newly admitted patients or patients that have been referred to dietetics for the first time this admission. When the Nutrition & Dietetic Department is fully staffed <u>all Dietitians</u> should aim to commence assessment of all referred patients within <u>1 working day</u> of receiving a referral. In times when clinical caseload is high and/or staff shortages referrals are prioritised as below, see appendix 1.
Review patients prioritisation	When the Nutrition & Dietetic Department is fully staffed <u>all Dietitians</u> should aim to review patients as required. In times when clinical caseload is high and/or there are staff shortages referrals are prioritised as below, see appendix 2.

7. Document owner

Director Allied Health

Enquiries relating to this guideline may be directed to:

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8. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
2	16/05/2024	16/05/2027	

9. Authorisation

This guideline has been authorised and issued by the Director Allied Health.

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	16/05/2024
Published date	17/05/2024

Appendix 1: Prioritisation guidelines for new adult inpatient referrals

Priority 1 – Immediate nutrition risk Seen within 24 hours or on the 1 st business day (if referral was made after hours or on a weekend)	Priority 2 – Nutrition intervention required prior to discharge Seen within 1-2 days or by end of 2 nd business day (if referral was made after hours or on a weekend)	Priority 3 – Nutrition intervention not essential prior to discharge Seen within 2-4 days if possible or referred to relevant outpatient service on discharge
New or unstable enteral or parenteral feeding Patients who are at risk of refeeding syndrome and commencing oral/enteral /parenteral feeding NBM >3 days Malnutrition Screening Tool score >3 Unintentional weight loss >5kg past month or during admission Eating Disorders (if presenting complaint/current issue) Newly diagnosed allergies/food intolerances (if current issues) New to dialysis or renal disease with potassium >6mmol/L High output stomas/fistulas Pressure injuries stage 3 or above Imminent discharge of priority 1 or 2 cases	Malnutrition Screening Tool score = 3 Unintentional weight loss 1-5kg during admission Disease with nutrition impact symptoms (nausea, vomiting, decreased appetite and/or diarrhoea) Post gastrointestinal surgery including ileostomy/colostomy Gastrointestinal disorders with impairment of nutritional status including pancreatitis, diverticulitis, acute inflammatory bowel disease, small bowel obstruction and liver disease Diabetes: newly diagnosed, unstable and/or newly commenced on insulin Poor wound healing Stage 2 pressure injury End Stage Liver Disease Residential care patient Patient on Home Enteral Nutrition	Pre-existing diabetes Pre-existing renal disease education without elevated potassium Pre-existing cardiac failure education Overweight/Obesity (Bariatric BMI>35) Dyslipidaemia Hypertension Constipation Diarrhoea without other nutrition impact symptoms Palliative care Newly diagnosed Cardiac Failure

Appendix 2 Table 2: Prioritisation Guidelines for review adult inpatients

Priority 1 Immediate nutrition risk Daily review	Priority 2 Regular monitoring required Review every second-third day	Priority 3 Stable patient Weekly to fortnightly review	Priority 4 Further inpatient intervention not required at present Review on request. Dietetic review not required prior discharge.
New, unstable enteral feeds (e.g. high gastric aspirates and/or electrolyte derangements), critically ill (ICU) patients Parenteral feeding Unstable patients with ongoing nutrition impact symptoms affecting medical outcomes (including Eating Disorder patients until management is established and nutrition risk is stabilised)	Stable patients on enteral feeds not at target rate. Oral feeding at risk of refeeding syndrome until electrolytes stabilise & on full intake/oral nutritional support Patients on non-nutritionally complete diets (e.g. Clear Fluids, Nourishing Fluids etc.) Unstable patients requiring oral nutritional support (i.e. may require enteral feeding, unstable electrolytes) High output ileostomy	Stable patients on established enteral feeds (weekly) Stable patients on oral nutritional support (7-14 days between reviews) Stable patients on therapeutic diets (e.g. High Energy, High Protein) awaiting placement Stabilised Eating Disorders	Stable patients who have been provided education, appropriate handover completed, and appropriate follow-up arranged Stable patient without oral nutritional support Stable patient established on a therapeutic diet Stable patients with no nutrition impact symptoms

Appendix 3 Table 3: Prioritisation guidelines for new paediatric inpatient referrals

Priority 1 – Immediate nutrition risk Seen within 24 hours or on the 1 st business day (if referral was made after hours or on a weekend)	Priority 2 – Nutrition intervention required prior discharge Seen within 1-2 days or by end of 2 nd business day (if referral was made after hours or on a weekend)	Priority 3 – Nutrition intervention not essential prior discharge Seen within 2-4 days if possible or referred to relevant outpatient service on discharge
Faltering growth (0 – 2 years of age) Feeding difficulty (0 – 2 years of age) Enteral feeding commencement and/or review New diagnosis of food allergy or intolerance, specifically cow's milk protein Maternal breastfeeding diet education on background of cow's milk protein intolerance Gastrointestinal symptoms where food allergy is suspected (0 – 2 years of age) Vegetarian diet education (0 – 1 year old) Obesity VLCD commencement/diet education <i>Paediatric nutrition screening tool: answer of "yes" to 2 or more questions – if/when commence at AHS</i>	Faltering growth (>2 years of age) Feeding difficulty with faltering growth (>2 years of age) Introduction to solids education Irritable bowel symptoms with faltering growth Overweight/obesity (0 – 2 year old) Vegetarian diet education (>2 years of age) Nutrient deficiencies (e.g. iron deficiency anaemia) Constipation and other gastrointestinal symptoms	Feeding difficulty without faltering growth (> 2 years of age) Irritable bowel symptoms without faltering growth (e.g. low FODMAP diet) Overweight and obesity (>2 year old) Fussy eating without any other co-morbidities

Appendix 4 Table 4: Prioritisation guidelines for review paediatric inpatients

Priority 1 – Immediate nutrition risk	Priority 2 – Regular monitoring required	Priority 3 – Stable patient	Priority 4 – Further inpatient intervention not required at present
Daily review	Review every second-third day	Weekly to fortnightly review	Review on request. Dietetic review not required prior discharge
Faltering growth (0 – 2 years of age) Feeding difficulty (0 – 2 years of age) Enteral feeding commencement and/or review New diagnosis of food allergy or intolerance, specifically cow's milk protein Maternal breastfeeding diet education on background of cow's milk protein intolerance Gastrointestinal symptoms where food allergy is suspected (0 – 2 years of age) Vegetarian diet education (0 – 1 year old) Unstable patients with ongoing nutrition impact symptoms affecting medical outcomes	Faltering growth (>2 years of age) Feeding difficulty with faltering growth (>2 years of age) Introduction to solids education Irritable bowel symptoms with faltering growth Overweight (0 – 2 year old) Vegetarian diet education (>2 years of age) Nutrient deficiencies (e.g. iron deficiency anaemia) Constipation and other gastrointestinal symptoms	Feeding difficulty without faltering growth (> 2 years of age) Irritable bowel symptoms without faltering growth (e.g. low FODMAP diet) Overweight and obesity (>2 year old) Fussy eating without any other co-morbidities	Stable patients who have been provided education, appropriate handover completed, and appropriate follow-up was arranged Stable patient without oral nutritional support Stable patient established on a therapeutic diet Stable patients with no nutrition impact symptoms

Note:

- 1) Category 1 review patients should take priority over Category 2 and 3 new referrals
- 2) Patients awaiting discharge should take priority over Category 2 & 3 New and Review patients